

Policy on Infection Prevention and Control in Clinical Areas

«Hospital Name»

Document control

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Date created	_____	Date of implementation	_____
Prepared by	Infection Prevention and Control Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» applies standard precautions in every clinical area at all times, adheres to hand-hygiene and transmission-based precaution guidelines, practices safe injection and infusion, and operates a documented antimicrobial usage policy with a stewardship programme that governs the ordering and monitoring of restricted antimicrobials.

1. Purpose

This policy explains how «Hospital Name» implements infection prevention and control processes in clinical areas: universal application of standard precautions, adherence to hand-hygiene and transmission-based precaution guidelines, safe injection and infusion practices, and an antimicrobial usage policy with a stewardship programme including a restricted antimicrobial ordering mechanism.

IPC processes for support services, HAI-prevention bundles for specific devices and procedures, and IPC surveillance are covered in separate hospital policies.

2. Scope

This policy applies to all clinical staff — doctors, nurses, allied health professionals, and students — working in any clinical area of «Hospital Name».

3. Policy standards

«Hospital Name» adheres to standard precautions in every clinical area at all times. International or national hand-hygiene guidelines are followed. Transmission-based precautions for contact, droplet and airborne modes are defined and used with appropriate PPE. Safe injection practice follows the one-needle-one-syringe-one-time rule. A documented antimicrobial usage policy — covering the type, monotherapy versus combination therapy, escalation and de-escalation, dose and duration, and a restricted list aligned with WHO AWaRe — is reviewed at least annually based on the organisation's site-specific antibiogram. Antimicrobials are prescribed only as per the policy; a defined mechanism governs ordering of restricted agents; deviations are brought to clinicians and CAPA documented; and the antimicrobial stewardship programme is monitored by a stewardship forum of clinicians.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not start any clinical procedure unless standard precautions — including appropriate PPE, sharps safety, and respiratory etiquette — are in place.
- Do not start an invasive step without having performed the required hand-hygiene moment for that step.
- Do not reuse a needle or syringe — one needle, one syringe, one time only.
- Do not start a restricted antimicrobial without following the organisation's antimicrobial stewardship path (documented indication and, where required, prior approval).
- Do not prescribe antimicrobials outside the organisation's policy.
- Do not operate without a current antimicrobial usage policy aligned with WHO AWaRe for the restricted list, reviewed at least annually.

5. What we do

5.1 Apply standard precautions at all times

Standard precautions are adhered to in every area of the organisation, at all times. Components include hand hygiene, appropriate use of PPE, respiratory etiquette, safe injection and infusion practices, use of sterile instruments and devices, cleaning and disinfection of environmental surfaces and equipment, and needle-stick and sharps injury prevention. Appropriate preparation of body parts before any procedure, and the use of disinfected or sterilised instruments, is ensured.

5.2 Adhere to hand-hygiene guidelines

«Hospital Name» adheres to international or national guidelines on hand hygiene. The WHO guidelines on hand hygiene in health care (2009) or the national equivalent serve as the

primary reference. Instructions are displayed near hand-washing areas. Compliance with hand-hygiene guidelines is monitored as part of the hospital's IPC surveillance programme.

5.3 Apply transmission-based precautions

Transmission-based precautions cover airborne, droplet and contact modes of transmission. PPE appropriate to each mode and situation is identified and used accordingly. These precautions are applicable across all areas of the organisation. CDC or equivalent guidelines are the reference for implementation.

5.4 Practice safe injection and infusion

Safe injection practice at all injection and infusion points follows the one-needle-one-syringe-one-time rule as recommended by CDC. Sharps are disposed of in puncture-proof containers at the point of use immediately after use. The WHO best-practices toolkit for injections and related procedures is a useful reference for the organisation's safe-injection guidance.

5.5 Establish and document the antimicrobial usage policy

«Hospital Name»'s antimicrobial usage policy identifies the clinical conditions in which antimicrobial agents — including anti-bacterial, anti-tubercular, anti-fungal, anti-viral and anti-parasitic agents — are used, in terms of: the type of antimicrobial agent; monotherapy versus combination therapy; escalation and de-escalation of therapy; and the dose and duration of therapy. The policy is based on the organisation's site-specific antibiogram and antimicrobial susceptibility data (culture-sensitivity based), updated at least annually. The policy identifies a list of restricted antimicrobial agents. The list of restricted antibiotics adheres to the WHO AWaRe classification.

5.6 Implement the antimicrobial stewardship programme

Antimicrobials are prescribed as per the organisation's antimicrobial usage policy. A defined mechanism exists for ordering restricted antimicrobial agents. Deviations from the policy are brought to the notice of the concerned clinician; corrective and preventive actions are taken and documented. The appropriate use of restricted agents is monitored. The antimicrobial stewardship programme is monitored by an antimicrobial stewardship forum of clinicians, and includes elements of leadership, accountability, drug expertise, action, tracking, reporting, and education. The organisation generates awareness among healthcare providers and the community on antimicrobial resistance and rational use of antimicrobials.

6. Stop-work authority

Do not start a clinical procedure when standard precautions are not in place (PPE, sharps safety, cough etiquette as the situation requires), when hand hygiene has not been done at the required moment, or when injection/infusion practice would reuse a needle or syringe.

Do not start a restricted antimicrobial without the organisation's antimicrobial-stewardship path (documented indication and, where required, approval).

Stop-work applies to the procedure or first dose start. Immediate life-saving care continues with the best available precautions and is documented.

The person who stops tells the treating doctor and the Infection Prevention and Control Officer the same shift. Refusing an unsafe start is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring clinical IPC processes are resourced and implemented across the organisation.
Infection Prevention and Control Officer	Owns implementation of this policy; leads the antimicrobial stewardship forum; receives stop-work escalations the same shift; brings CAPA and audit findings to the IPC committee.
Infection Prevention and Control Nurse(s)	Monitors compliance with standard precautions, hand hygiene, transmission-based precautions, and safe injection; raises stop-work when a trigger is observed.
Antimicrobial Stewardship Forum	Monitors the antimicrobial stewardship programme; reviews restricted-agent orders and deviations; oversees the site-specific antibiogram update.
Nursing Superintendent	Ensures nursing staff in all clinical areas follow standard and transmission-based precautions, hand hygiene, and safe-injection practice.
Quality Coordinator	Audits this policy; holds training records and staff acknowledgements; logs stop-work events.

8. Quality monitoring

Monitoring area	What is monitored
Standard precautions compliance	IPC round records confirming standard precautions in use in all clinical areas; audit results tabled at IPC committee.
Hand-hygiene compliance	Monthly compliance monitoring of hand-hygiene compliance; results shared with relevant staff and tabled at IPC committee.
Transmission-based precaution compliance	Audit records confirming PPE appropriate to each mode is used and available.

Safe injection compliance	Audit records confirming one-needle-one-syringe-one-time practice; needle-stick injury reports reviewed.
Antimicrobial usage policy	Current policy reviewed at least annually; restricted list aligned with WHO AWaRe; site-specific antibiogram updated.
Antimicrobial stewardship	Restricted-agent orders tracked; deviation records with CAPA documented; stewardship forum meeting minutes.
Stop-work events	Stop-work events logged with trigger, action taken, and outcome.

9. Training and staff acknowledgement

All clinical staff shall be familiar with standard precautions, hand-hygiene guidelines, transmission-based precautions, safe injection practice, the antimicrobial usage policy, the restricted-agent ordering mechanism, and the stop-work authority in this policy.

I have read the Policy on Infection Prevention and Control in Clinical Areas of «Hospital Name». I will follow the processes described, including the stop-work authority in Section 6.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all clinical staff, the IPC Officer, IPC Nurse(s), the antimicrobial stewardship forum, department heads, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
AMS	Antimicrobial Stewardship
AMR	Antimicrobial Resistance
AWaRe	Access, Watch, Reserve (WHO antibiotic classification)
CAPA	Corrective and Preventive Action
CDC	Centers for Disease Control and Prevention
IPC	Infection Prevention and Control
IPCO	Infection Prevention and Control Officer
IPCN	Infection Prevention and Control Nurse
NABH	National Accreditation Board for Hospitals and Healthcare Providers
PPE	Personal Protective Equipment
WHO	World Health Organization

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
IPC.3.a	CORE*	Sections 3, 5.1 and 6 address universal application of standard precautions including PPE, sharps safety, respiratory etiquette, sterile instruments, and surface cleaning; stop-work triggers when precautions are not in place.
IPC.3.b	CORE*	Sections 3, 5.2 and 6 address adherence to hand-hygiene guidelines; stop-work trigger when required hand-hygiene moment is missed before an invasive step.
IPC.3.c	Commitment*	Sections 3 and 5.3 address transmission-based precautions (airborne, droplet, contact), with PPE identified for each mode, applicable across the organisation.
IPC.3.d	CORE*	Sections 3, 5.4 and 6 address the one-needle-one-syringe-one-time safe injection rule; stop-work trigger when this rule would be violated.
IPC.3.e	Commitment*	Sections 3 and 5.5 address the documented antimicrobial usage policy covering type, mono/combination therapy, escalation/de-escalation, dose, duration, and a restricted list aligned with WHO AWaRe, reviewed at least annually.
IPC.3.f	CORE	Sections 3, 5.6 and 6 address the AMS programme: prescribing per policy, restricted-agent

		ordering mechanism, deviation CAPA, stewardship forum, and the seven AMS programme elements; stop-work trigger for restricted antimicrobial without the stewardship path.
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13. Required Records/Evidence Checklist

Standard precautions — IPC.3.a (CORE*)

- Written standard-precautions guidance (or section within IPC Manual) covering all components: hand hygiene, PPE, respiratory etiquette, safe injection, sterile instruments, environmental cleaning, and sharps injury prevention.
- IPC round or audit records confirming standard precautions are in use in all clinical areas.
- Staff training records on standard precautions.

Hand hygiene — IPC.3.b (CORE*)

- Named hand-hygiene guideline adopted (e.g., WHO 2009 or national equivalent).
- Monthly hand-hygiene compliance monitoring records.
- Evidence of display of hand-hygiene instructions near washbasins.

Transmission-based precautions — IPC.3.c (Commitment*)

- Written transmission-based precaution guidance covering contact, droplet, and airborne modes with PPE specification for each.
- Audit records confirming appropriate PPE use by mode in clinical areas.

Safe injection — IPC.3.d (CORE*)

- Written safe-injection and infusion guidance naming the one-needle-one-syringe-one-time rule.
- Audit records and needle-stick injury log reviewed for reuse events.
- Records confirming puncture-proof sharps containers are at the point of use.

Antimicrobial usage policy — IPC.3.e (Commitment*)

- Dated antimicrobial usage policy covering all five required content elements: type, mono/combination, escalation/de-escalation, dose, and duration.
- Restricted antimicrobial list aligned with WHO AWaRe.
- Site-specific antibiogram/culture-sensitivity data used for the policy update.
- Annual review record confirming the policy remains current.

Antimicrobial stewardship programme — IPC.3.f (CORE)

- Restricted-agent ordering mechanism (forms, electronic approvals, or equivalent) and records of its use.
- Deviation records with CAPA documented and tracked to closure.
- Antimicrobial stewardship forum meeting minutes.
- Stop-work event log with trigger, action, and outcome for each event.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. IPC.3.
2. WHO Guidelines on Hand Hygiene in Health Care, 2009. World Health Organization.
3. WHO AWaRe (Access, Watch, Reserve) Antibiotic Book. World Health Organization.
4. CDC Safe Injection Practices: One Needle, One Syringe, Only One Time.

Disclaimer

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